

8-12 WEEKS

BEGINNER FRIENDLY

SIMPLE PROTOCOL

FULL PCT

BEGINNER ANABOLIC STEROID CYCLE FULL GUIDE

8-12 Week Foundation Protocol — Safe, Simple & Effective

AMAN TRAINING CLUB • Beginner Series • 2024 Edition

AMAN TRAINING CLUB

■ **MEDICAL DISCLAIMER:** For educational purposes only. Always consult a licensed physician before beginning any hormonal protocol. Anabolic steroids are controlled in many countries.

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■ 1. WHY TESTOSTERONE ONLY — THE GOLDEN RULE

Every beginner's first cycle should be **Testosterone only**. This is the most important rule in bodybuilding pharmacology. Here's why:

- Testosterone is the base of ALL steroid cycles — your body already makes it
- If side effects occur, you know EXACTLY which compound caused them
- Beginners respond incredibly well to testosterone alone — no need to complicate it
- Adding multiple compounds before knowing your individual response is dangerous
- Test-only cycles are predictable, manageable, and reversible with proper PCT
- Expect 15–25 lbs of lean mass on a first Test cycle (with proper training + diet)

■ 2. CORE COMPOUND

Compound	Short Name	Weekly Dose	Injection Schedule	Duration
Testosterone Enanthate	Test E	300–500 mg	2 × weekly (Monday/Thursday)	8–12 weeks
Testosterone Cypionate	Test C	300–500 mg	2 × weekly (alternative)	8–12 weeks

Dosage recommendation by experience level:

Experience Level	Recommended Dose	Notes
True Beginner (1st cycle)	300 mg/week	Minimal side effects, excellent gains
Intermediate Beginner (2nd cycle)	400 mg/week	Balanced risk/reward ratio
Advanced Beginner (3rd+ cycle)	500 mg/week	Maximum beginner gains

■ 3. OPTIONAL KICK-STARTER: DIANABOL (WEEKS 1–6)

Dianabol can be added to accelerate early gains while Test E/C builds up in your system (long esters take 3–4 weeks to reach peak blood levels). This is optional — not required.

- **Dose:** 20–30 mg/day (split into 2–3 doses with meals)
- **Duration:** Weeks 1–6 only
- Expect rapid weight gain (5–10 lbs in 2–3 weeks) — mostly water + muscle
- Water retention will drop after stopping Dbol — this is normal
- Increase AI dose during Dbol weeks due to higher aromatization
- Requires liver support: TUDCA 500 mg/day throughout Dbol use
- **Not recommended for true first-timers** — run Test only first

■ 4. 8 WEEK vs 12 WEEK PROTOCOL

Protocol	Stack	Notes
8 Weeks	300–400 mg Test E/C + optional Dbol wks 1–4	Shorter recovery, less suppression, ideal for 1st cycle
10 Weeks	400–500 mg Test E/C + optional Dbol wks 1–6	Good balance — recommended for most beginners
12 Weeks	400–500 mg Test E/C + optional Dbol wks 1–6	Maximum gains, longer recovery needed post-cycle

■ 5. AROMATASE INHIBITOR (AI)

Testosterone converts to estrogen via aromatase. **Some estrogen is necessary** for muscle growth, mood, and libido. The goal is to **keep estrogen in range** — not eliminate it.

Option A: Arimidex (Anastrozole) — Recommended

- 0.5 mg every other day (EOD) as a starting dose
- Reduce to 0.25 mg EOD if experiencing low-estrogen symptoms (joint pain, low libido, low mood)
- Available as pill; exact dosing is easy

Option B: Aromasin (Exemestane)

- 12.5–25 mg every other day
- Suicidal AI — permanently deactivates aromatase enzyme (more difficult to titrate)
- Better choice for men prone to high estrogen

Signs of HIGH estrogen: Water retention, gynecomastia (puffy/sensitive nipples), mood swings, high blood pressure

Signs of LOW estrogen: Joint pain, low libido, depression, fatigue, poor erections

■ 6. POST CYCLE THERAPY (PCT)

PCT is non-negotiable. Starting PCT at the wrong time is a common mistake. For Test Enanthate/Cypionate (long esters): Begin PCT **14–18 days** after your last injection.

Compound	Dose	Timing	Purpose
Clomid (Clomiphene Citrate)	50 mg/day	Weeks 1–3 of PCT	Primary LH/FSH stimulator
Nolvadex (Tamoxifen Citrate)	20 mg/day	Weeks 1–4 of PCT	Anti-estrogen, SERM
HCG (optional but recommended)	500 IU EOD	2 weeks BEFORE PCT	Testicular size recovery

After PCT, get blood work (testosterone, LH, FSH, E2). If levels have recovered to normal range, PCT is complete. If suppressed, consult a doctor (TRT may be needed in rare cases).

■ 7. ESSENTIAL SUPPLEMENT STACK

- **Milk Thistle / TUDCA:** 500–800 mg/day for liver protection (especially with Dbol)
- **Omega-3 Fish Oil:** 3–4 g/day for cholesterol management and cardiovascular health
- **Vitamin D3 + K2:** 5000 IU D3 + 100 mcg K2 daily
- **Zinc + Magnesium (ZMA):** Nightly — supports testosterone production in PCT
- **Creatine Monohydrate:** 5 g/day — amplifies strength and muscle gains
- **Multivitamin:** High-quality daily multivitamin
- **Whey Protein:** To hit protein targets (minimum 1 g/lb body weight)
- **Blood pressure monitor:** Check BP twice weekly — AAS can raise BP

■ 8. BEGINNER TRAINING PROGRAM

Recommended splits for beginners on cycle:

Day	Focus	Key Exercises
Monday	Chest + Triceps	Bench Press, Incline DB Press, Cable Flyes, Tricep Dips
Tuesday	Back + Biceps	Deadlifts, Pull-Ups, Barbell Rows, Bicep Curls
Wednesday	REST / Light Cardio	30 min walk or LISS cardio
Thursday	Shoulders	OHP, Lateral Raises, Front Raises, Face Pulls
Friday	Legs	Squats, Leg Press, Romanian Deadlift, Calf Raises
Saturday	Arms + Abs	Isolation work, Core circuit
Sunday	REST	Active recovery, stretching

■ 9. BEGINNER NUTRITION GUIDE

- **Find your TDEE** (Total Daily Energy Expenditure) using an online calculator
- **Bulking:** Eat +300–400 kcal above TDEE for lean gaining
- **Protein:** Minimum 1 g/lb body weight (higher = better, up to 2 g/lb)
- **Carbohydrates:** Rice, oats, potatoes, fruits — complex carbs are your energy source
- **Fats:** Eggs, olive oil, avocado, nuts — don't go below 0.3 g/lb body weight
- **Water:** 3–4 litres minimum daily
- **Meal timing:** Pre-workout meal (2 hrs before) + post-workout meal (within 1 hr after)

■ 10. SIDE EFFECTS — WHAT TO WATCH FOR

Side Effect	Likelihood	Management
Acne (back/shoulders)	Common	Good hygiene, salicylic acid wash, reduce dose
Hair loss/thinning	Genetic	Finasteride or Nizoral shampoo; accept risk if predisposed
Water retention	Common	Reduce sodium, manage AI dose, drink more water
Gynecomastia	Manageable	Start Nolvadex 20 mg/day immediately; add AI
High blood pressure	Monitor	Monitor BP; reduce dose; add Hawthorn Berry supplement
Testicular atrophy	Expected	Normal on cycle; HCG prevents it; reverses post-PCT
Mood changes/aggression	Possible	Usually dose-dependent; communicate with people close to you
Libido changes	Common	Often improves ON cycle; may dip during PCT — temporary

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